

HP FAMOTIDINE FILM-COATED TABLET 20 MG

VIFAMxx-reg4

DESCRIPTION

Square, light orange film-coated tablet, bevel-edged with shallow convex faces.

COMPOSITION

Each tablet contains:
Famotidine 20 mg.

PHARMACODYNAMICS

Famotidine is an effective competitive H_2 receptor antagonist, the effect of which is particularly clearly concentrated on H_2 receptors. It reduces the concentration and amount of acid and pepsin of the gastric juices in the basal and stimulated secretion.

The effect of oral administration of famotidine is rapid. The effect of famotidine is long lasting when recommended doses are used, and it is effective with relatively low concentrations in the blood. The duration of effect, plasma concentration and secretion in the urine are dose dependent.

Oral administration of famotidine leads to the antacid effect starting within an hour of administration. The peak effect is dose dependent and it is achieved within 1-3 hours of administration.

PHARMACOKINETICS

Absorption

Famotidine is rapidly absorbed. Peak plasma concentration is dose dependent and is achieved within 1-3 hours of administration. The kinetics of famotidine are linear. The bioavailability of oral famotidine is 40-45% on average. Food contained in the stomach will not affect the bioavailability. First pass metabolism of famotidine is minor. Repeat administration will not cause accumulation of the drug in the body.

Distribution

The binding of famotidine in plasma proteins is relatively small (15-20%). The plasma half-life is about 3 hours both after oral single doses and during repeated administration (5 days).

Biotransformation

Up to 30-35 % of famotidine is metabolised in the liver to an inactive sulfoxide metabolite.

Elimination

When administered orally, an average of 65-70% of the absorbed famotidine is excreted in the urine. About 25-30% of the total oral dose is excreted unchanged in the urine. The renal clearance of famotidine is 250-450 ml/min, so consequently, excretion also takes place through tubular secretion. A small proportion may be secreted as sulfoxide.

Pharmacokinetics in different patient groups

The average elimination half-life of famotidine in plasma was prolonged to 11.7 hours in patients with creatinine clearance of 30ml/min or less. Patients with maximum creatinine clearance of 10ml/min had an average plasma half-life of approximately 13 hours. The elimination half-life in these patients may exceed 20 hours. In patients with anuria the elimination half-life was prolonged to about 24 hours. In patients with creatinine clearance of 30 ml/min or less, only 21.2% of the intravenous injection was excreted in the urine.

Disturbance of liver function apparently has no effect on the pharmacokinetics of famotidine. Age-related impairment of renal function should be considered when determining the dose.

INDICATIONS

Famotidine is indicated in the short-term treatment of duodenal ulcer, benign gastric ulcer and hypersecretory conditions such as Zollinger-Ellison syndrome. It is also used in the prevention of relapses of duodenal ulceration, symptomatic relief of gastro-oesophageal reflux disease and healing of oesophageal erosions or ulceration associated with gastro-oesophageal reflux disease.

CONTRAINDICATIONS

Hypersensitivity to the active substance or to any of the excipients of the product.

WARNINGS AND PRECAUTIONS

Since cross sensitivity has been observed among H_2 -receptor antagonists, famotidine should not be administered to patients with a history of hypersensitivity to other drugs in this class.

Gastric neoplasm: Before starting treatment of gastric ulcer with famotidine, the possibility of malignant gastric tumour should be excluded. The symptomatic response of a gastric ulcer to famotidine therapy does not exclude the possible existence of a malignant tumour.

Do not administer famotidine tablets in cases of minor gastro-intestinal complaints.

In patients with duodenal ulcers and benign gastric ulcers the *H. Pylori* status should be determined. Wherever possible, patients with *H. Pylori* should undergo eradication therapy to eliminate the bacteria.

Renal insufficiency: Since famotidine is secreted mainly via the kidneys, caution should be exercised when treating patients with renal insufficiency. A reduction in the daily dose should be considered if creatinine clearance falls below 10 ml/min.

Paediatric population: In the children are not established safety and efficacy.

Use in the elderly: No dosage adjustment is required based on age alone. As elderly patients are more likely to have decreased clearance of famotidine, care should be taken in dose selection and it may be useful to monitor renal function.

General: In case of long-term treatment with high dosage, monitoring of blood count and liver function is recommended.

In case of long-standing ulcer disease, abrupt withdrawal after symptom relief should be avoided.

Ability to drive and using machines: Some patients have experienced adverse reactions such as dizziness and headache while taking famotidine. Patients should be informed that they should avoid driving vehicles or operating machinery or doing activities which require prompt vigilance if they experience these symptoms.

INTERACTIONS WITH OTHER MEDICAMENTS

During concomitant use of drugs where the absorption of the drug is affected by the amount of gastric juice, a possible effect on the absorption should be taken into consideration. The absorption of ketoconazole and itraconazole could be reduced. Ketoconazole should be given two hours before famotidine administration.

Probenecid inhibits renal tubular secretion of famotidine, and has been shown to cause a 50% increase in plasma levels of famotidine. Therefore, concomitant use of probenecid and famotidine should be avoided.

Concomitant use of famotidine and antacids could reduce the famotidine uptake and cause lower plasma levels of famotidine. Therefore, famotidine should be administered 1-2 hours before antacids.

Concomitant use of sucralfate inhibits absorption of famotidine. Therefore, sucralfate should not be administered within 2 hours after famotidine.

Famotidine does not interact with the cytochrome P450-linked drug metabolizing enzyme system. Compounds metabolized by this system which have been tested in man have included warfarin, theophylline, phenytoin, diazepam, propranolol, aminopyrine and antipyrine. Indocyanine green as an index of hepatic blood flow and/or hepatic drug extraction has been tested and no significant effects have been found.

Alterations of gastric pH may affect the bioavailability of certain drugs resulting in a decrease in the absorption of atazanavir.

Risk of loss of efficacy of calcium carbonate when co-administered as phosphate binder with famotidine in hemodialysis patients.

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PREGNANCY AND LACTATION

Pregnancy: Famotidine is not recommended for use in pregnancy, and should be prescribed only if clearly needed. Before a decision is made to use famotidine during pregnancy, the physician should weigh the potential benefits from the drug against the possible risks involved.

Lactation: Famotidine is excreted in breast milk. Breast-feeding mothers should either stop using this drug or stop breast-feeding, since there is a possibility of famotidine affecting the infant's gastric acid secretion.

SIDE EFFECTSBlood and the lymphatic system disorders

Very rare

Thrombocytopenia, Leukopenia, Agranulocytosis, Pancytopenia, Neutropenia.

Psychiatric disorders

Very rare

Reversible psychic disturbances including Hallucinations, Disorientation, Confusion, Anxiety disorders, Agitation, Depression, Reduced libido, Insomnia.

Nervous system disorders

Common

Headache, Dizziness

Uncommon

Taste disorder

Very rare

Paraesthesia, Somnolence, Epileptic seizures, convulsions; grand mal seizures (particularly in patients with impaired renal function).

Gastrointestinal disorders

Common

Constipation, Diarrhoea

Uncommon

Nausea, Vomiting, Abdominal discomfort or distension, Flatulence, Dry mouth

Hepato-biliary disorders

Rare

Intrahepatic cholestasis (visible sign: jaundice).

Very rare

Hepatitis; cholestatic jaundice, Increase in liver enzyme abnormalities (transaminases, gamma GT, alkalinephosphatase, bilirubin)

Metabolism and nutrition disorders

Uncommon

Loss of appetite (anorexia)

Skin and subcutaneous tissue disorders

Uncommon

Rash, Pruritus, Urticaria

Very rare

Alopecia, Stevens Johnson syndrome/toxic epidermal necrolysis sometimes fatal

Immune system disorders

Very rare

Hypersensitivity reactions (angioneurotic oedema, anaphylaxis, bronchospasm)

Respiratory, thoracic and mediastinal disorders

Very rare

Interstitial pneumonia sometimes fatal

Musculoskeletal, connective tissue and bone disorders

Very rare

Muscle cramps, Arthralgia

Cardiac disorders

Very rare

AV block with H₂ receptor antagonists administered intravenously

General disorders and administration site conditions

Uncommon

Fatigue

Very rare

Chest tightness

Reproductive system and breast disorders

Very rare

Impotence

SYMPTOMS OF OVERDOSE AND TREATMENT

In the event of overdose the aim should be to remove any unabsorbed drug from the alimentary tract with the usual measures from the gastrointestinal tract, clinical monitoring, and supportive therapy should be employed.

RECOMMENDED DOSEUsual adult and adolescent dose:Duodenal ulcer:

Oral, 40 mg once daily at bedtime or 20 mg 2 times daily.

Prophylaxis of recurrent duodenal ulcer:

Oral, 20 mg at bedtime.

Gastric ulcer:

Oral, 40 mg once daily at bedtime.

Gastric hypersecretory conditions (eg. Zollinger-Ellison syndrome):

Oral, 20 mg every 6 hours, the dosage being adjusted as needed and therapy continued for as long as clinically indicated. Doses up to 160 mg every 6 hours have been administered to some patients with severe Zollinger- Ellison syndrome.

Gastro-oesophageal reflux:

Oral, 20 mg 2 times daily for up to 6 weeks. The recommended oral dose for oesophagitis due to gastro-oesophageal reflux disease is 20 to 40 mg 2 times daily for up to 12 weeks.

Dosage adjustment for patients with moderate to severe renal insufficiency:

Dosage adjustment is required for patients with moderate to severe renal insufficiency. Since CNS adverse effects have been reported in patients with moderate to severe renal insufficiency, to avoid excess accumulation of the drug, the dose of famotidine may be reduced to half the recommended dose or the dosing interval may be prolonged to 36 – 48 hours as indicated by the patient's clinical response.

Note: The information given here is limited. For further information, please consult your doctor or pharmacist.

Storage:

Store below 30°C. Protect from light and moisture.

Presentation/Packing:

Blisters of 10 x 10's

Product Registration Holder: Hovid Pharmacy Sdn. Bhd.

121, Jalan Tunku Abdul Rahman,
30010 Ipoh, Perak, Malaysia.

Manufactured by: HOVID Bhd.

Lot 56442, 7 1/2 Miles, Jalan Ipoh/Chemor,
31200 Chemor, Perak, Malaysia.

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